sanofi



- Diagnosed with T2DM 7 years ago
- Failed to achieve lasting control on basal insulin (currently on 26 U) + OAD
- High FPG & High PPG
- Struggles with weight gain and hypoglycemia

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Powerful Glycemic Control

HbA1c

Superior HbA1c reduction with once-daily iGlarLixi from baseline vs. twice-daily **premix insulin** (BIAsp 30)[1#]

iGlarLixi
1.3± 0.1%

BIAsp 30
1.1± 0.1%

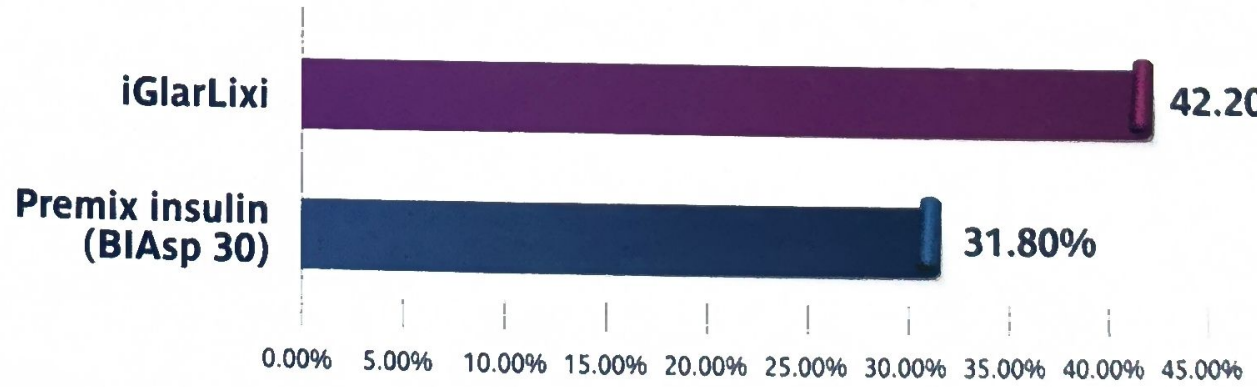
(95% CI: -0.4, -0.1) $p < 0.001$

2x

With **SOLIQUA®** ~2x greater **HbA1c reduction vs. basal insulin** (iGlar 100 U/mL)$p<0.0001$ from **baseline to Week 30**[2^]

More patients achieved HbA1c <7% with once-daily **iGlarLixi from baseline** vs. twice-daily **premix insulin** (BIAsp 30)[1]

Number of patients achieving HbA1c <7%



PERCENTAGE OF PATIENTS
(OR 1.65 [95% CI: 1.25 to 2.19])

Lower Risk of Hypoglycemia

Lower rate of any level of hypoglycemia with once-daily **iGlarLixi from baseline** vs.twice-daily **premix insulin** (BIAsp 30)[1]

55% Lower risk of level-2 hypoglycemia (ADA definition: <54 mg/dL) with once-daily **iGlarLixi from baseline** *vs.* twice-daily **premix insulin** (BIAsp 30) [OR (95% CI): 0.45 (0.28, 0.73)][1]

Weight Benefit

Change in weight from the baseline while switching from basal insulin[1,2,5]



SOLIQUA® achieved **superior HbA1c reduction** ($p<0.001$), **superior weight change** ($p<0.001$), and **less hypoglycemia** (OR 0.62; 95% CI 0.47, 0.81)[1]

vs. premix insulin BIAsp30 *

SOLIQUA®

insulin glargine (100 U/mL) & lixisenatide

EXPERIENCE THE SOLIQUA® TOUCH

Powerful efficacy[1,2,3]

Low rates of hypoglycemia[1]



Weight neutral[1]

Easy start[4]

Abbreviations: GLP-1 RA: glucagon-like peptide 1 receptor agonist; BIAsp 30: biphasic aspart insulin 30 (30% insulin aspart and 70% insulin aspart protamine); T2DM: type 2 diabetes mellitus; OADs: oral antidiabetic drugs; FPG: fasting plasma glucose; PPG: postprandial glucose; iGlar:insulin glargine; OR: Odds ratio; CI: Confidence interval; BB: Basal bolus, BI: Basal Insulin, BID: Twice daily; BMI: Body mass index; GGD2: GetGoal Duo-2; LS: Least squares; PSM: Propensity matching score; QD: Once daily; SE: Standard error; SGLT-2: Sodium-glucose transport protein 2; TID: Three times daily; HbA1c: Glycated hemoglobin
iGlarLixi is an fixed-ratio combination FRC of basal insulin glargine 100 units/mL (iGlar) and the short-acting GLP-1 RA lixisenatide (Lixi). premix insulin (biphasic insulin aspart 30 (BIAsp 30))

[1]Adjusted OR of iGlarLixi vs BIAsp 30 with associated two-sided CI (at the specified significance level that is passed from family 1 of the primary objectives), calculated by logistic regression model adjusted for fixed categorical effects of randomization strata (basal insulin dose at screening <30 units and ≥30 units and SGLT2 inhibitor use [yes, no] at screening) and treatment group as well as fixed continuous covariates of baseline values for each of the primary endpoints (HbA1c and body weight) | *From baseline to Week 26. | *In adults with T2DM insufficiently controlled on basal insulin + OADs | * When switching from basal insulin in patients insufficiently controlled on basal insulin (+ metformin +/- OADs) | [5]SoliMix was a 26-week open-label, multicenter, randomized trial comparing iGlarLixi (SOLIQUA®) vs premix insulin analog (BIAsp 70/30) in adults with suboptimally controlled T2D (HbA1c ≥7.5–≤10%) despite receiving stable doses of BI + OADs (metformin ± SGLT-2 inhibitor) for 3 months. Primary efficacy endpoints were non-inferiority in HbA1c reduction (margin 0.3 %) or superiority in bodyweight change for iGlarLixi vs BIAsp 30. Primary results showed that switching to QD iGlarLixi resulted in better glucose control (change in HbA1c, -1.3 ± 0.1 % with SOLIQUA® vs -1.1 ± 0.1 % with BIAsp 30, non-inferiority: LS mean difference -0.2% [97.5% CI: -0.4, -0.1], p<0.001, and superiority: LS mean difference -0.2% [95% CI: -0.4, -0.1], p<0.001) with weight benefit -0.7 ± 0.2 kg with SOLIQUA® vs 1.2 ± 0.2 kg with BIAsp 30, (LS mean difference -1.9 kg [p<0.001]) and less overall hypoglycemia 31.2% (n=138) with SOLIQUA® vs 42.4% (n=187) with BIAsp 30, (OR 0.62 [95% CI: 0.47, 0.81]) vs BID BIAsp 70/30.2 Overall safety and tolerability profiles of SOLIQUA® and premix insulin BIAsp 30 were consistent with the known safety profile of each product. Gastrointestinal disorders were more common in the SOLIQUA® group vs premix insulin: nausea 7.7% vs 0.0%, vomiting 1.1% vs 0.2%, and dyspepsia 0.9% vs 0.2%, respectively. | [6] This is a post-hoc propensity-score-matching analysis which compared the efficacy and safety of SOLIQUA® and a BB regimen in patients with T2DM uncontrolled on BI using data from two randomized controlled trials: LixiLan-L and GGD2. From the LixiLan-L study, 195 patients who had been randomized to SOLIQUA® were matched based on age, sex, race, T2DM duration, baseline BMI, HbA1c, FPG, insulin dose, and metformin use with 195 patients from the GGD2 trial who had been randomized to BB. BB regimen in GGD2 involved insulin glargine 100 units/mL + insulin glulisine either QD or TID. Of the 195 patients included in this analysis, 49% (n=95) were on TID and the remainder on QD for insulin glulisine. Primary and co-primary endpoints were change in HbA1c for LixiLan-L, and change in HbA1c and body weight for GGD2, over the treatment period. To avoid confounding from longer exposure in LixiLan-L, HbA1c and weight changes were assessed at 24 weeks for SOLIQUA® (LixiLan-L) and 26 weeks for BB (GGD2). All p-values are nominal without multiplicity adjustment. The primary endpoint of HbA1c change showed a reduction by a LS mean (SE) of 1.02% (0.07) in patients in the iGlarLixi group, while it was reduced by 0.74% (0.07) in the BB group. The estimated LS mean treatment difference in HbA1c decline over the study was -0.28% (SE 0.08; 95% CI -0.43 to -0.13, p=0.0002) in favor of iGlarLixi. Safety: The rate of hypoglycemia events per patient year was higher with BB compared with SOLIQUA®; overall, the estimated rate ratio of BB to SOLIQUA® was 2.85 (p<0.0001), while the rate ratio for nocturnal events was 4.32 (p<0.0001). | †Mixed-effect model with repeated measures.

References:
1. Rosenstock J, Emral R, Sauque-Reyna L, *et al.* Advancing therapy in suboptimally controlled basal insulin-treated type 2 diabetes: Clinical outcomes with iGlarLixi versus premix BIAsp 30 in the SoliMix randomized controlled trial. *Diabetes Care.* 2021;44(10):2361–2370 | 2. Aroda VR, Rosenstock J, Wysham C, *et al.* Efficacy and Safety of LixiLan, a Titratable Fixed-Ratio Combination of Insulin Glargine Plus Lixisenatide in Type 2 Diabetes Inadequately Controlled on Basal Insulin and Metformin: The LixiLan-L Randomized Trial. *Diabetes Care.* 2016 Nov;39(11):1972-1980. | 3. Rosenstock J, Aronson R, Grunberger G, *et al.* Benefits of LixiLan, a titratable fixed-ratio combination of insulin glargine plus lixisenatide, versus insulin glargine and lixisenatide monocomponents in type 2 diabetes inadequately controlled on oral agents: The LixiLan-O randomized trial. *Diabetes Care.* 2016;39(11):2026–2035. | 4. SOLIQUA® SoloStar® PI.pdf (sanofi.in) accessed on 13 February 2024 | 5. Tabák ÁG, et al. *Diabetes Ther.* 2020;11:305-318.

Abridged Prescribing Information - Fixed Ratio Combination of Insulin Glargine and Lixisenatide 100U + 50mcg /33mcg. **SOLIQUA® SoloStar® Composition:** SOLIQUA® Solostar®100U + 50mcg /33mcg contains per mL 100 units insulin glargine and 50/33 mcg of lixisenatide. **Indication:** SOLIQUA® is indicated for the treatment of adults patients with Obesity with insufficiently controlled type 2 diabetes mellitus to improve glycemic control as an adjunct to diet and exercise in addition to metformin with or without SGLT2 inhibitors, when this has not been provided by metformin alone or metformin combined with another oral glucose lowering medicinal product (sulfonylurea, glinide, DPP-4 inhibitors or gliptins, and Sodium-glucose co- transporter 2 (SGLT2) inhibitors or gliflozins) or with basal insulin or with glucagon-like peptide-1 (GLP-1) receptor agonist. **Dosage and administration:** The starting daily dose of SOLIQUA® is 10 units (10 units of insulin glargine and 5mcg of lixisenatide) and the maximum daily dose of SOLIQUA® is 60 units of SOLIQUA® (60 units insulin glargine and 20 mcg lixisenatide). SOLIQUA® should be administered subcutaneously once a day within 1 hour prior to any meal. It is preferable that the prandial injection of SOLIQUA® is performed before the same meal every day, when the most convenient meal has been chosen. If a dose of SOLIQUA® is missed, it should be injected within the hour prior to the next meal. The dose of SOLIQUA® must be individualized based on clinical response and is titrated based on the patient's need for insulin. The lixisenatide dose is increased or decreased along with insulin glargine dose and also depends on which pen is used. SOLIQUA® is titratable and available in two pens (10-40) pen or (30-60) pen, providing different dosing options. SOLIQUA® is to be injected subcutaneously in the abdomen, deltoid, or thigh. The injection sites should be rotated within the same region (abdomen, deltoid, or thigh) from one injection to the next in order to reduce the risk of lipodystrophy and cutaneous amyloidosis. Patients should be instructed to always use a new needle.
Safety related Information - Special populations: In pediatric patients below the age of 18 years, safety and effectiveness have not been established. SOLIQUA® can be used in elderly ≥ 65 years patient provided dose is adjusted on individual basis, based on glucose monitoring. In patient ≥ 75 years of age, therapeutic experience is limited. Frequent glucose monitoring and dose adjustment may be necessary for SOLIQUA® in patients with hepatic and renal impairment. **Contraindications:** Hypersensitivity to the active substances or to any of the excipients. **Warnings and precautions:** SOLIQUA® should not be used in patients with type 1 diabetes mellitus or for the treatment of diabetic ketoacidosis. There have been few reported events of acute pancreatitis with lixisenatide, causal relationship not established. Inform patients of characteristic symptoms; persistent, severe abdominal pain. Discontinue SOLIQUA® if pancreatitis suspected; lixisenatide should not be restarted if acute pancreatitis confirmed. Caution in history of pancreatitis. Instruct patients to continuously rotate injection site to reduce risk of lipodystrophy and cutaneous amyloidosis; delayed insulin absorption and worsened glycemic control may occur after injection at affected sites. Sudden change in injection site to unaffected area has resulted in hypoglycemia; blood glucose monitoring is recommended after changing injection site and dose adjustment of antidiabetic medications may be considered. Hypoglycemia may occur if the dose of SOLIQUA® is higher than required, dose must be individualised based on clinical response and patient's need for insulin. SOLIQUA® not recommended in patients with severe gastrointestinal disease, including severe gastroparesis. Not recommended in severe renal impairment or end-stage renal disease. Caution in patients receiving oral medicinal products that require rapid gastrointestinal absorption, careful clinical monitoring or have a narrow therapeutic ratio. Advise patients to take precautions to avoid fluid depletion. In rare cases the presence of antibodies may necessitate adjustment of SOLIQUA® dose. **Interactions:** No interaction studies with SOLIQUA® have been performed. For detailed information, please refer to the interaction section in the prescribing information. **Effects on ability to drive:** Patients should be advised to take precautions to avoid hypoglycemia while driving and using machines. Pregnancy and Lactation: SOLIQUA® should not be used during pregnancy or breast-feeding. Undesirable effects: The most frequently reported adverse reactions during treatment with SOLIQUA® were hypoglycemia and gastrointestinal adverse reactions. Very common: hypoglycemia. Common: dizziness, nausea, diarrhoea, vomiting.
For full prescribing information please contact: Sanofi Healthcare India Pvt. Ltd., Sanofi House, CTS No. 117-B, L&T Business Park, Saki Vihar Road, Powai, Mumbai - 400072
Updated: Jan 2023. **Source:** Insulin Glargine + Lixisenatide CCDS v6 dated 29th Sept 2022

For full Abridged Prescribing Information of Soliqua, visit here: Soliqua®SoloStar® API pdf (sanofi.in)
Disclaimer: For the use of registered endocrinologists or specialists in internal medicine only

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